

TATA MEDICAL CENTER KOLKATA

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NABL ACCREDITED & NABH HEALTHCARE
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Report ID: TDM-TAC-2026-3392
Barcode: TDM*0914*33928

TACROLIMUS (FK506) WHOLE BLOOD TROUGH LEVEL (LC-MS/MS)

Patient Name:	Debabrata Mukherjee	Date of Exam:	September 14, 2026
MRN / Patient ID:	TMCK-BMT-2026-4411	Age / Gender:	39 Yrs / Male
Blood Group & Rh:	O+ (Rh Positive)	Referring Doctor:	Dr. Saurabh Bhawe, MD, DM (Clinical Hematology)
Clinical Diagnosis:	Allogeneic BMT Day +45 (GVHD Immunosuppression Trough)	Specimen / Source:	Whole Blood (K2-EDTA Pre-Morning Dose Trough C0)

Drug Assay Parameter	Patient Trough Level	Target Therapeutic Window	Unit	Clinical Action
Tacrolimus Level (LC-MS/MS)	7.4	5.0 - 10.0 (Day 30-90)	ng/mL	Within Optimal Therapeutic Window
Collection Timing	12.0 Hours Post-Dose	11.5 - 12.5 Hours	Hours	Valid Steady-State C0 Trough
Current Oral Dose	1.5 mg q12h (3 mg/day)	Titrated Dose	Oral BID	Optimal Bioavailability
Serum Creatinine Concordance	0.98	0.70 - 1.30	mg/dL	Zero Nephrotoxic Impact
Signs of Acute GVHD	None Reported (Grade 0)	Target: No GVHD	Clinical Exam	Immunosuppression Adequate

CLINICAL INTERPRETATION & RECOMMENDATIONS:

- Liquid Chromatography-Tandem Mass Spectrometry (LC-MS/MS) confirms Tacrolimus level of 7.4 ng/mL. - Trough concentration is in the ideal therapeutic zone for Day +45 post-allogeneic BMT (target 5.0 - 10.0 ng/mL). - No evidence of toxic accumulation (neurotoxicity/tremor absent) or subtherapeutic graft-versus-host vulnerability. - Recommendation: Maintain oral Tacrolimus at 1.5 mg every 12 hours; re-check trough in 14 days.
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